

Course Code and Name: MSJ3262-Health Communication

Project Name: Communication Campaign

Semester: Spring 2025

Project Description: For MSJ 3262: Health Communication at the University of Liberal Arts Bangladesh (ULAB), I developed a health communication intervention plan titled “Breaking the Silence: A Communication Intervention Plan to Address Adolescent Mental Health in Bangladesh.” The project focused on raising awareness of depression and anxiety among adolescents, reducing mental health stigma, and encouraging early recognition of stress-related symptoms. Using secondary research and a systematic health communication planning process, I designed SMART objectives and a multi-channel communication strategy targeting students aged 10–18. The project proposed mental health workshops and social media awareness campaigns for schools, providing a practical framework that can support educators, NGOs, and policymakers in promoting adolescent mental well-being across Bangladesh.

Project Justification: This project focuses on addressing adolescent mental health in Bangladesh by raising awareness of depression, anxiety, and the importance of early support. I chose this topic because academic pressure, social stigma, and limited access to mental health services prevent many students from seeking help. The project emphasizes the role of communication in reducing stigma, encouraging open discussions, and promoting positive mental health among young people. It proposes a culturally appropriate communication strategy that supports collaboration among schools, parents, media, and community organizations. Through this work, I hope to contribute to improving mental health awareness, encouraging early intervention, and creating a more supportive environment for adolescents across Bangladesh.

Background and Problem Statement

Prevalence and Impact

Depression and Anxiety: In Dhaka, studies have shown that 26.5% of adolescents suffer from moderate to severe depression, and 18.1% experience anxiety. Factors like lack of sleep and smoking are associated with depression, while anxiety is linked to age and social relationships. Mental health issues among adolescents in Bangladesh often go underreported due to stigma, lack of awareness, and a shortage of mental health professionals. Disorders such as depression, anxiety, and stress are on the rise, partly due to social and academic pressures. This information found from, Islam, M. S., Rahman, M. E., Moonajilin, M. S., & Van Os, J. (2021).

National Summary: According to the National Mental Health Survey of Bangladesh (2019), the overall prevalence of mental disorders among individuals aged 10 and above is 13.6%. This highlights a major public health concern, especially in schools where academic demands, peer relationships, and family expectations add pressure.

Gender Disparity: In Dhaka, 19% of male and 30% of female school-going adolescents report symptoms of depression.

Contributing Factors

Academic Pressure: Intense competition and exam stress significantly contribute to mental health problems. Children in Bangladesh face extreme academic pressure, often with little time for recreational activities. This leads to stress and anxiety, with many students reporting feelings of being overwhelmed and inadequate.

Lack of Awareness: Limited understanding of mental health among students, teachers, and parents worsens the issue. Changes in family structure, financial constraints, and social expectations also contribute to adolescent mental health concerns. Additionally, gender norms often influence how mental health is perceived, with female students being more at risk of anxiety and depression.

Stigma: Cultural stigma surrounding mental health prevents open discussions and help-seeking. In many parts of Bangladesh, mental health remains a taboo subject, and families and communities are hesitant to talk about or seek help for mental health issues. This stigma delays early intervention and treatment.

Target Audience Analysis

Primary Audience: School-aged children (ages 10–18) in both urban and rural areas. Early intervention is crucial for this age group. Mental health issues that begin in adolescence can

continue into adulthood, affecting academic performance, social relationships, and overall well-being.

Challenges: Academic stress, bullying, family expectations, peer pressure, and lack of mental health education.

Needs: Education on mental health, safe spaces to talk about feelings, coping strategies, and access to counselors or mental health resources.

Secondary Audience: Parents, teachers, and school counselors. These groups are in the best position to identify signs of mental health problems in children and offer early support.

Challenges: Lack of understanding about mental health issues, which are sometimes viewed as weakness rather than medical conditions.

Needs: Increased knowledge on how to support children facing mental health challenges, reduce stigma, and create a more open environment for emotional expression.

Tertiary Audience: Community leaders and local healthcare providers. These groups can help shift cultural perceptions of mental health, raise awareness, and support youth in accessing services.

Challenges: Limited availability of mental health professionals in rural areas, lack of resources, and cultural norms that prevent open discussion about mental health.

Needs: Training to identify mental health issues and refer children to appropriate services, along with building community-level support networks.

Communication Objectives

1. **Specific:** Raise awareness and reduce stigma related to mental health issues among adolescents in both urban and rural school environments in Bangladesh.
2. **Measurable:** Increase awareness of the symptoms and signs of mental health disorders by 30%, as measured through pre- and post-intervention surveys.
3. **Achievable:** Develop and distribute educational materials and conduct workshops in at least 50 schools across Bangladesh over the next six months.
4. **Relevant:** Address the growing mental health crisis among Bangladeshi youth, focusing on the prevention of anxiety, depression, and stress, and promoting early intervention.

5. **Time-bound:** Implement the intervention and complete evaluation within six months.

Key Messages and Communication Channels

Key Messages:

Mental Health Awareness: “Mental health is a part of our overall well-being. It affects how we think, feel, and act. Just like physical health, taking care of our mental health is important.”

Recognizing Symptoms: “Know the signs of stress, anxiety, and depression. If you’re feeling sad, worried, or overwhelmed for a long time, it’s important to talk to someone. These feelings are common, and help is available.”

Seeking Help: “Seeking help is a sign of strength. Talk to someone you trust whether it’s a teacher, parent, or counselor.”

Breaking the Stigma: “Mental health issues are common and treatable. Mental health problems are just as real as physical ones. Anyone can face mental health challenges, and it’s okay to talk about it.”

Communication Channels

Workshops and Seminars: Organize school-based sessions with mental health professionals. Conduct in-person workshops in both urban and rural schools led by trained mental health experts. These sessions will teach students how to recognize signs of mental health problems and where to seek help.

Social Media Campaign: Use platforms like Facebook and Instagram to reach a wider audience. Share mental health tips, personal stories, and professional advice through platforms such as Facebook, Instagram, and YouTube. Collaborate with popular influencers and local celebrities to spread the message.

Printed Materials: Distribute brochures, posters, and leaflets in schools and community centers with information about mental health awareness, symptoms, and available support services.

Radio and TV Programs: Broadcast discussions on mental health to reach rural areas. Use radio and TV to deliver educational content about mental health to adolescents and their families in areas with limited internet access. These programs should include expert interviews, success stories, and general tips.

Implementation Strategy and Timeline

Phase 1: Preparation (Month 1)

Content Development: Create culturally appropriate materials (brochures, videos, posters) focused on the importance of mental health, coping strategies, and breaking stigma.

Training Facilitators: Train school counselors, teachers, and health professionals on delivering the intervention with a focus on empathetic communication and mental health literacy.

Partnerships: Collaborate with mental health organizations and local influencers to build credibility and support for the initiative.

Phase 2: Implementation (Months 2–4)

Workshops: Conduct mental health workshops in 50 schools focusing on interactive activities, role-plays, and real-life examples.

Launch Social Media Campaign: Start a campaign using local influencers to spread key mental health messages and encourage conversations among youth.

Broadcast Radio/TV Programs: Begin airing educational programs on mental health. Ensure rural communities are included in the outreach.

Phase 3: Evaluation (Month 5)

Surveys: Conduct pre- and post-workshop surveys in schools to measure changes in knowledge and attitudes toward mental health.

Focus Groups: Organize focus group discussions with students and teachers to gather feedback on the effectiveness of the intervention and areas for improvement.

Phase 4: Reporting (Month 6)

Reporting: Prepare a detailed evaluation report based on surveys and feedback. Share findings with stakeholders such as local government, NGOs, and schools to encourage continued support for mental health education.

Monitoring and Evaluation Plan

Pre- and Post-Intervention Surveys: Assess changes in knowledge and attitudes before and after the intervention. These surveys will measure changes in students' and teachers' knowledge, attitudes, and behavior.

Focus Group Discussions: Gather qualitative insights about participants' experiences. Use these sessions to understand the deeper impact of the intervention and whether stigma has been reduced.

Behavioral Observation: Track the number of students seeking help through school counselors or mental health services to evaluate changes in help-seeking behavior.

Feedback Forms: Collect responses from teachers, students, and parents.

Regular Check-ins: Monitor the implementation process through periodic meetings.

Budget Consideration

Educational Materials – 50,000 BDT

Facilitator Fees – 100,000 BDT

Media Campaign – 75,000 BDT

Supplies and Transportation – 25,000 BDT

Miscellaneous – 20,000 BDT

Total: 270,000 BDT

Cultural and Ethical Considerations

Cultural Sensitivity: Ensure all materials and messages are culturally appropriate and resonate with local communities. Use local language and examples that reflect the Bangladeshi context.

Confidentiality: Maintain the privacy of individuals sharing personal experiences. Ensure all information collected from students, teachers, and parents remains confidential, especially sensitive details related to mental health.

Inclusivity: Ensure the intervention is accessible to students from various socio-economic backgrounds and adapt materials as needed for students with disabilities.

Reference:

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Project Learnings and Outcomes: This project strengthened my research, planning, and health communication skills by teaching me how to develop a comprehensive communication intervention plan for a public health issue. It improved my ability to conduct audience analysis, set SMART objectives, design communication strategies, and prepare monitoring and evaluation plans. Through this work, I gained a deeper understanding of adolescent mental health, behavior change theories, and the role of communication in reducing stigma and promoting positive health behaviors. The project also enhanced my critical thinking by helping me analyze cultural, social, and gender-related factors that influence mental health awareness and help-seeking behavior. Overall, this experience made me a more confident and responsible communication planner, capable of designing evidence-based, culturally appropriate health communication campaigns that support meaningful and sustainable behavior change.